

Description

A woman arrives to the Southern Midwest Medical Center's Emergency Department in active labor, and delivers in the emergency department at 7:30 AM on February 1, 2025. The newborn has a Congenital diaphragmatic hernia (CDH), and has suffered Meconium aspiration syndrome (MAS). A clerical assistant registers the newborn as a patient. She records the date of birth, race, ethnicity, residence, insurance information, and health history. The baby has not yet been given a name and is subsequently updated once provided by the mother. The clerical assistant also enters the patient's chief complaint as, "Emergency Room birth with Congenital diaphragmatic hernia (CDH) (ICD-10 Q79.0)". Upon examination, the physician determines the newborn should be admitted to the hospital NICU and put on ECMO to stabilize the baby before performing surgery to repair the CDH (0BQT0ZZ Repair Diaphragm, Open Approach). The newborn is discharged home after recovering from surgery. The Southern Midwest Medical Center reports syndromic surveillance data to the state health department (SHD). At 7:45 AM on February 1, 2025, the hospital's electronic health record module for syndromic surveillance data assembles and transmits a Registration message about this encounter to SHD.

This Test Case provides an example of an ED visit for which the patient's chief complaint is captured as free-text, working diagnosis and final diagnosis are captured with SNOMED CT codes, the patient is discharged from the ED and admitted for inpatient care, and the Admit/Encounter Reason is captured with a SNOMED CT code.

This test case involves 4 steps:

- a registration message (ADT^A04),
- an update message (ADT^A08),
- a discharge message (ADT^A03)
- an admit message (ADT^A01).

Messages in this test case will be used in a Manual Test Step under functional requirement testing for validating the HIT capability of populating Visit ID (PV1-19.1) and Visit Time (PV1-44.1) each with the same value for all messages included in this Test Case.

Comments

No Comments

Pre-condition

No PreCondition

Post-Condition

No PostCondition

Test Objectives

This test case examines a Health IT Module's ability to create an ADT^A04-Registration message, an ADT^A08-Update message, an ADT^A03-Discharge message, and an ADT^A01-Admission message within HL7 Version 2.5.1 Implementation Guide: Syndromic Surveillance, Release 1 - US Realm Standard for Trial Use July 2019, conformance requirements.

The following test objectives and variants are supported by this test case:

- Setting is Emergency Department with Inpatient Admission.
- Chief complaint is captured as narrative in PV2-3 Admit Reason.
- **Diagnoses are submitted test using either ICD-10-CM or SNOMED-CT:** Per ASTP/ONC, for certification testing a Health IT Module can use either the ICD-10-CM or SNOMED CT® value sets in the submitted messages for all of the test steps in the Syndromic Surveillance Test Suite. Where the tool does not have test data that supports the Health IT Module's value set (either ICD-10-CM or SNOMED CT®), the developer of the Health IT Module must provide the codes and testers must perform a visual inspection of the messages for these test steps to ensure that equivalent and valid ICD-10-CM or SNOMED CT® are used to populate the messages. This policy is conformant to the named standard HL7 Version 2.5.1 Implementation Guide: Syndromic Surveillance, Release 1 - US Realm Standard for Trial Use July 2019). SNOMED-CT® codes are provided/required in the Test Data for coding the diagnoses in this Test Case.
- **Patient Name is Identifiable in the submissions:** Per ASTP/ONC, for certification testing HIT developers must demonstrate that their system supports the Patient Name and pseudonymized data in PID-5 (this policy is conformant to the named standard HL7 Version 2.5.1 Implementation Guide: Syndromic Surveillance, Release 1 - US Realm Standard for Trial Use July 2019). The patient name presented in an identified manner is required for this Test Step.
- Multiple Races.
- Pediatric case where age is measured in months.
- Updates to patient demographics
- Updates to Diagnoses.
- Discharge Disposition Inpatient Admission followed by Home.
- Insurance is unknown

Evaluation Criteria

No evaluation criteria

Notes

No Note